

# Hair Across a Lifetime

*Hormonal seasons, shedding, thinning, and the grace of hair that is allowed to change*

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## Learning Objectives

- Explain how pregnancy, postpartum, and menopause each affect the hair growth cycle
- Distinguish normal daily shedding from telogen effluvium and from androgenetic alopecia
- Explain the biology of greying and why it is not a sign of hair "weakening"
- Respond to a client's hair-life-stage concern with accuracy and reassurance rather than alarm

Hair keeps a record of the body

Return to Module One's growth cycle. What that module presented as a steady, ongoing process is, across a lifetime, anything but static — it responds continuously to hormones, stress, illness, and age. A practitioner who understands these seasons can offer a client something more valuable than a product: an accurate explanation for what her hair is doing, and the reassurance that most of it is normal.

Pregnancy: the extended anagen phase

Rising oestrogen during pregnancy extends the anagen (growth) phase and holds more strands there than usual, which is why hair often appears fuller and glossier during pregnancy — fewer hairs than normal are moving into the resting and shedding phases at any given time. It is a genuine, hormonally driven state, not a myth.

Postpartum: telogen effluvium

The reversal comes after birth. As hormone levels fall back toward baseline, the large cohort of strands that had been "held" in anagen during pregnancy move into telogen and shed together, typically beginning two to four months after delivery. This synchronized shedding, called telogen effluvium, can feel alarming — clients often describe handfuls of hair in the shower — but it is a temporary, self-resolving return to baseline, not a sign of illness, and it typically settles within six to twelve months.

#### Menopause and hormonal thinning

As oestrogen declines during and after menopause, its normally protective effect on hair follicles lessens, while the relative influence of androgens (male-pattern hormones present in all bodies) increases. This shift can lead to female pattern hair loss — a gradual widening of the parting and diffuse thinning at the crown, distinct from the patchy loss of alopecia areata covered in Module Three. It is common, it is manageable with the right medical guidance, and it is never something to minimise or brush past in a consultation.

#### Androgenetic alopecia across genders

The same androgen-sensitivity mechanism drives male pattern baldness, typically progressing along a predictable pattern from the temples and crown, often beginning as early as the twenties. Genetics, not "poor haircare," is the primary driver in both patterns — a fact worth stating plainly to any client carrying quiet guilt about it.

#### Greying: a different story than damage

Hair goes grey when the melanocyte stem cells that supply pigment to the follicle gradually deplete over time — a process influenced heavily by genetics, and modestly by oxidative stress. It is not a sign that the strand itself has weakened; grey hair is structurally the same as pigmented hair, though it can feel coarser or wirier due to a slightly different growth pattern and reduced natural oil coverage as the scalp ages. Reassure clients accordingly: greying reflects a change in pigment production, not a change in the strength of the hair itself.

#### When normal shedding becomes a signal

Module One's figure of 50–100 shed hairs a day remains the baseline for normal exogen shedding at any life stage. What changes with hormonal seasons is the rate, not the mechanism — a client shedding visibly more than usual for a sustained period, outside the expected postpartum or menopausal windows, is a candidate for the referral conversation you learned in Module Three, not a candidate for a new shampoo.

## Key Terms

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[object Object]

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## Practical Activity · The Life-Stage Script

Write a short, warm consultation script — three to four sentences — you could use with a postpartum client concerned about shedding, and a separate script for a client entering menopause noticing thinning. Read both aloud to a partner and refine until they sound reassuring, not clinical.